جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : اميره عبدالحكيم طه احمد
المحافظة : الشرقيه **الحي / المدينة :** الزقازيق **العنوان :** //
رقم البطاقة : 26412271300969 **رقم التليفون :** 01017000800 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 27-12-1964 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اميره عبدالحكيم طه احمد
National ID: 26412271300969

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





قيمادكاريلبوواا

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT Form

Patient ID:
Name :
Sex: Female
Medical number: 83067188
Date: 5/31/2024
MDT number:
Patient Age : 60 Y

Family history | Phone: 01017000800 | DMI | DR | REASON OF MDT
Surgical status | NRS Score | GROUP | Date of MDT: 03/06/2024
Comorbidity | Menopausal status | Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :
CTCAP--> NEWLY DEVELOPED LT UPER LOBE LUNG MASS AND PLEURAL EFFUSION

ER	PR	Her 2 Neu	SISH	Ki67
7/8	7/8	2	Amplified	20

N	M

Mammography

PATIENT HISTORY

CASE OF MBC TO PLEURAL & MED LNS,,RECEIVED 6 CYCLES OF TAXOL/HERCEPTIN THEN MAINTANNACE AI/ HERCEPTIN FOR ONE YEAR
NOW NEW CTCAP AND PETCT REVEALED NEWLY DEVELOPED LUNG NODULES AND REACCUMLATION OF PLEURAL EFFUSION DENOTING PD
/2023 DP--> shifted to tykerb/xeloda x 12 cycles till 2/2024
/2024 HFS (dt toxicity) xeloda stopped & conyinued on tykerb / aromasine till 7/2024
ren refused by MOH
st ct cap: SD

MDT OUTCOME & PLAN

- Aromasin & Tykerb

SIGNATURE

ZOICAL ONCOLOGY		RADIOLOGY	
IRGERY		PATHOLOGY	
ychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: امیره عبد الحکیم طه	
MR. : 3515	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was placed on Xeloda 2 × 2 for management of hand-foot syndrome and a new CT evaluation was ordered.

The patient, a 61-year-old individual with metastatic breast cancer involving lung, pleura, and lymph nodes since 2021, has tumor markers ER 7/8, PR 7/8, HER2 +2 with SISH amplification, and Ki-67 20 %. Six cycles of Taxol/Herceptin were administered, followed by maintenance Herceptin for one year; CT in May 2023 demonstrated progressive disease. Subsequent therapy with Tykerb/Xeloda continued until February 2024, when Xeloda was stopped due to grade III–IV hand-foot syndrome. The regimen was then switched to Tykerb/Aromasin until July 2024, but Tykerb approval was rejected by the Ministry of Health despite stable disease on CT in May 2024. The last cycle of Tykerb/Aromasin was given in January 2025; from January 2025 to September 2025 the treatment line is unclear. CT in September 2025 showed progressive disease, prompting a trial to resume Tykerb/Xeloda, which was again rejected by the Ministry of Health. The patient is currently on Xeloda, with improvement of hand-foot syndrome. For cycle 6 of Xeloda 3 × 3, labs were obtained and a CT is planned for today.

The patient presented today for Xeloda single-agent therapy prescribed as 3 × 3 × 14, pending laboratory results.

A CT performed in March 2026 demonstrated a reduction of the pulmonary nodule to 9 mm from 12 mm, persistent pleural effusion with underlying lower-lobe collapse, and otherwise unremarkable findings; the patient remains eligible for Xeloda after laboratory review.

The patient was seen for Xeloda 3 × 2 × 14 and reported grade 2 foot syndrome; the next clinic visit is scheduled for 16 July 2026.

On the most recent assessment, a CT of the chest, abdomen, and pelvis performed in July 2026 showed stable disease while on Xeloda 3 × 2 × 14; the next visit is planned for 27 August 2026. The patient has completed three cycles of Xeloda.

تحريرا في ,

التاريخ :	20-09-2026	**الوقت :**	
الطبيب المعالج :	نهي سعيد علي محمد		

يعتمد ,

Patient Name: Amira Abd Al Hakeem Taha Ahmed	Date : 25/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 60 Year(s)
Medical ID: 5361/2021	Pat. ID: 13525

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Left breast soft tissue mass lesion measuring 33 x 15 mm showed microcalcifications
- Marked left pleural effusion with subsequent near total collapse of the left upper and lower lobes.
- Right upper lobar small nodule measuring 8 mm.
- No hilar or mediastinal masses or lymph node enlargement.
- No right pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Enlarged pre-aortic and porta hepatis lymph nodes with the largest measures 20 x 15 mm.
- No ascites.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Amira Abd Al Hakeem Taha Ahmed	Date : 25/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 60 Year(s)
Medical ID: 5361/2021	Pat. ID: 13525

Opinion:

- Known case of left breast cancer.
- Left marked pleural effusion with underlying near total collapse of the left upper and lower lobes.
- Right upper lobar nodule.
- Multiple abdominal lymph nodes.

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

القصر العيني

وحدة فحوص أمراض الثدي

كلية الطب البيطري

IMMUNOHISTOCHEMISTRY REPORT

Name: أميرة عبد الحكيم طه احمد حسب الله

Pathology Number: 428i - 2021

Age: 57 y Date: 17/11/2021

Patient's I.D.: 26412271300969

CLINICAL DATA: A case of *left* breast cancer, for estrogen, progesterone receptors, Her-2/neu and KI-67.

PROCEDURE: Sections from paraffin block were treated with antigen retrieval solution and then treated with monoclonal antibodies for estrogen, progesterone & Her-2/neu, (CONFIRM™ anti-Estrogen Receptor (ER) (SP1) Rabbit & CONFIRM™ anti-Progesterone Receptor (PR) (1E2), CONFIRM™ anti-HER-2/neu (4B5) Primary Antibody), and CONFIRM anti-Ki67 (30-9) Primary Antibody), using Ventana iView™ system. DAB was used as a chromogen and hematoxylin as a counter stain.

RESULTS:

Examination of section treated for estrogen receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.

Examination of section treated for progesterone receptors revealed strong positive nuclear staining in most of tumor cells by chromogen.

Examination of section treated for Her-2/neu revealed strong incomplete membrane staining in >10% of tumor cells by chromogen (score 2+).

Examination of section treated for Ki67 revealed positive nuclear stain in average of ±35% of tumor cells.

CONCLUSION:

Tumor Cells Were:

- **Positive** For Estrogen Receptors, Score 8/8.
- **Positive** For Progesterone Receptors, Score 8/8.
- **Equivocal** For Her-2/neu (Score 2+).
- **35%** KI-67 Proliferative Index

COMMENT: All control's show appropriate reactivity.

PROF. DR. AHMED ABDEL AZIZ
PROF. DR. SAMIA MOHAMED GABAL

أ.د/ احمد عبد العزيز
أ.د/ سامية محمد جبل

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name: Amira Abd Al Hakeem Taha Ahmed	Date : 25/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 60 Year(s)
Medical ID: 5361/2021	Pat. ID: 13525

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- Marked left pleural effusion with subsequent near total collapse of the left upper and lower lobes.
- Right upper lobar small nodule measuring 8 mm.
- No hilar or mediastinal masses or lymph node enlargement.
- No right pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Enlarged pre-aortic and porta hepatis lymph nodes with the largest measures 20 x 15 mm.
- No ascites.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Amira Abd Al Hakeem Taha Ahmed	Date : 25/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 60 Year(s)
Medical ID: 5361/2021	Pat. ID: 13525

Opinion:

- Known case of left breast cancer.
- Left marked pleural effusion with underlying near total collapse of the left upper and lower lobes.
- Right upper lobar nodule.
- Multiple abdominal lymph nodes.

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m8090

اسم المريض :	اميره عبدالحكيم طه	النوع :	Female
السن :	58 Y	الجنسية :	المصرية
تاريخ اخذ العينة :		تاريخ استلام العينة :	29/11/2021 12:52
طالب الفحص :	مركز اورام دار السلام	تاريخ التقرير :	06/12/2021 10:34
عنوان :	مركز اورام دار السلام		

Primary Sample :
Paffin Block
Test :
ER&PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)
Comment :
Technique :
mple recieved was treated by : CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-
gesterone receptor(PR) (1E2)Rabbit monoclonal primary antibody.
mple recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection
n Ventana benchmark GX.
mple recived was treated by : CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.
Result :
moderate nuclear staining of > 60 % of tumour cells (core biopsy)
moderate nuclear staining of > 60 % of tumour cells (core biopsy)
2/neu protein over expression : moderate continuous membrane staining of > 10 % of tumor cells (core biopsy).
positive nuclear staining of 10 - 20 % of tumour cells (core biopsy)
Referal Lab :
Comment :
Conclusion :
POSITIVE (SCORE 7/8)
POSITIVE (SCORE 7/8)
R/NEU PROTEIN OVER EXPRESSION : EQUIVOCAL (SCORE 2+)
; THE PROLIFERATION INDEX OF THE TUMOUR IS 10 - 20 %

Pathologist: د/توريا ماهر

Lab Director: د/توريا ماهر

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pathologycht@yahoo.com
ook: مركز اورام دار السلام

Immunohistochemistry
[[DOCTYPE:IHC]]

Cancer Center

Nasser Institute Hospital

مستشفيات أورام

Patient Name: [redacted] Date: 10/05/2023

Referring Physician: Prof. Dr.

- *Metabolically active FDG avid left upper lung lobe epically speculated mass lesion, measuring 2.x2x1.6cm with max. SUV~4.8 for follow up*
- *There are no pleural or pericardial abnormalities.*
- *Physiologic FDG uptake is noted in the myocardium.*

Abdomen & Pelvis:

- *The liver is average in size showing normal FDG uptake with no metabolically active FDG avid focal lesion.*
- *The spleen is average in size showing normal FDG uptake with no metabolically active FDG avid splenic focal lesion.*
- *Subcentimetric metabolically inactive abdominal pelvic, L.Ns (reactive).*
- *No metabolically active lymph nodes are seen.*
- *Physiologic FDG excretion is seen in the kidneys & bladder.*

Musculoskeletal:

- *Normal FDG activity is seen along the rest of the surveyed skeleton. No sclerotic or lytic lesions are noted on CT.*

OPINION:

- ***Positive PET/CT study for metabolically active FDG avid left upper lung lobe soft tissue speculated mass lesion with metabolically active FDG avid mediastinal nodal lesion.***
- ***Low grade FDG left breast central mass lesion as mentioned***

Thank you for your reference

PET/CT Unit

Radiology consultant

DR /Heba Nabil , MD

Revised by

Dr/ Mohamed Emam

PET Scan
[[DOCTYPE:PET_SCAN]]